

Eligibility checklist

A subject is eligible only when every inclusion row is Yes, every exclusion row is No, each answer has a source document behind it, and the investigator has signed.

Screening number	Date of consent	Protocol version	Checklist version
		Amendment 3, June 17, 2019	Version 1

How to complete this checklist

- Complete it at Visit 1. Repeat it at Visit 3 before anyone opens IWRS to randomize. Repeat it at Visit 9 for extension entry, where every criterion applies except seizure frequency.
- You gather and verify the source. Only the PI or a delegated sub-investigator decides eligibility and signs.
- In each answer cell write Y or N, your initials and the date of the source you checked. "Per subject" is acceptable only where the source column says interview.
- A lab value that excludes the subject at Visit 1 may be repeated once at an unscheduled visit for confirmation. File both reports.
- There are no waivers. A trivial exception needs the medical monitor's written approval before randomization. File the approval behind this checklist.
- One re-screen is allowed. The subject keeps the same screening number and the two screenings are told apart by date.
- Criteria wording below is condensed. Check it line by line against Sections 7.3 and 7.4 of your IRB-approved protocol before first use.

Inclusion criteria

All 10 must be Yes. Criterion 6 can only be answered at Visit 3, after the 8-week baseline.

#	Criterion	Verify against	Visit 1	Visit 3
1	Male or female, age 18 to 70 years inclusive	Date of birth in the medical record or on photo ID. Age on the date of consent	Y / N	Y / N
2	Weight at least 40 kg	Weight measured at the visit, not reported	Y / N	Y / N
3	Written informed consent signed by the subject or legal guardian before entering the study. If the	Signed and dated consent form. Consent note showing the time,	Y / N	Y / N

#	Criterion	Verify against	Visit 1	Visit 3
	guardian consents because the subject cannot, the subject's written or verbal assent is also obtained	before any study procedure		
4	Diagnosis of partial epilepsy by the ILAE Classification of Epileptic Seizures, established by clinical history and an EEG consistent with localization-related epilepsy. A normal interictal EEG is allowed when the clinical history meets the diagnosis	Neurology notes. EEG report filed in source. Approved Diagnostic Review Form before randomization	Y / N	Y / N
5	Uncontrolled partial seizures despite treatment with at least 1 AED within approximately the last 2 years	AED history in the medical record	Y / N	Y / N
6	During the 8-week baseline: at least 8 partial seizures, counting only simple partial with motor component, complex partial or secondarily generalized. No seizure-free interval longer than 25 days. At least 3 of these seizures in each of	Seizure diary and the baseline seizure count table below	Not yet	Y / N

#	Criterion	Verify against	Visit 1	Visit 3
	the two consecutive 4-week segments			
7a	Stable doses of 1 to 3 AEDs for at least 4 weeks before Visit 1, to continue unchanged throughout the study	Medication list, prescribing notes, pharmacy fills	Y / N	Y / N
7b	Vagal nerve stimulator, if present: not counted as an AED. Implanted at least 5 months before Visit 1. Parameters stable for at least 4 weeks before baseline	Implant record. Latest device interrogation report	Y / N / NA	Y / N / NA
7c	Benzodiazepines taken at least once a week in the month before Visit 1, for epilepsy, anxiety or sleep, count as 1 AED and continue unchanged. Then at most 2 other AEDs are allowed	Medication list	Y / N / NA	Y / N / NA
8	CT or MRI within the past 10 years that ruled out a progressive cause of epilepsy. If none exists, one is performed before randomization	Imaging report with date, filed in source	Y / N	Y / N

#	Criterion	Verify against	Visit 1	Visit 3
9	Subject can be reached by telephone	Number confirmed at Visit 1	Y / N	Y / N
10	Female of childbearing potential uses an acceptable form of birth control	Method and start date documented. See the list below	Y / N / NA	Y / N / NA

Acceptable birth control (Section 7.5)

- Hormonal contraception for at least 3 months before Visit 1, combined with a barrier method. Hormonal alone is not adequate. Synthetic progestins are acceptable; natural progesterone is not.
- Intrauterine device placed at least 3 months before screening.
- Diaphragm with spermicide, cervical cap, or condoms with contraceptive gel, foam or cream.
- Tubal ligation, or partner's vasectomy, at least 6 months before screening.
- Postmenopausal, defined as no menses for at least 12 months.
- Abstinence. If the subject becomes sexually active, one of the methods above is required.

Not of childbearing potential means postmenopausal or permanently sterilized, for example tubal occlusion, hysterectomy or bilateral salpingectomy.

Exclusion criteria

All 31 must be No. Rows 8, 9, 21, 22, 24 wait for the central lab report and row 23 waits for the investigator's ECG read, so Visit 1 eligibility is never final on the day.

#	Criterion	Verify against	Visit 1	Visit 3
1	History of serious systemic disease, including hepatic insufficiency, renal insufficiency, a malignant neoplasm, any disorder with survival prognosis under 3 months, or any disorder the investigator judges places the subject at excessive risk	Medical history, problem list, investigator assessment	Y / N	Y / N

#	Criterion	Verify against	Visit 1	Visit 3
2	History of nonepileptic or psychogenic seizures	Neurology notes, prior video-EEG reports	Y / N	Y / N
3	Only nonmotor simple partial seizures, or primary generalized epilepsies	Seizure classification, Diagnostic Review Form	Y / N	Y / N
4	Seizure clusters within 3 months before Visit 1: episodes under 30 minutes in which seizures are so frequent that individual seizures cannot be told apart	Seizure history, subject and caregiver interview	Y / N	Y / N
5	Lennox-Gastaut syndrome, present or past	Medical history	Y / N	Y / N
6	Epilepsy surgery scheduled within 8 months after Visit 1	Neurology and surgical notes, interview	Y / N	Y / N
7	Pregnancy or lactation	Serum pregnancy test at Visit 1, urine test at Visit 3, interview	Y / N	Y / N
8	Any clinically significant laboratory abnormality the investigator judges exclusionary	Central lab report, signed and dated by the investigator with each abnormal value assessed	Y / N	Y / N
9	Significant active hepatic disease: AST or ALT	Central lab report	Y / N	Y / N

#	Criterion	Verify against	Visit 1	Visit 3
	above 2 times the upper limit of normal, or total or direct bilirubin outside normal limits			
10	Active CNS infection, demyelinating disease, degenerative neurologic disease, or any CNS disease deemed progressive during the study that may confound interpretation	Medical history, imaging report	Y / N	Y / N
11	Clinically significant psychiatric illness, or psychological or behavioral problems the investigator judges would interfere with participation	Medical history, investigator assessment	Y / N	Y / N
12	Psychotic disorders or unstable recurrent affective disorders evident by antipsychotic use. Major depressive episode now or within the past 6 months	Medical history, medication list	Y / N	Y / N
13	Intermittent rescue benzodiazepines more than once in the month before	Medication history, pharmacy fills, interview	Y / N	Y / N

#	Criterion	Verify against	Visit 1	Visit 3
	Visit 1. One or two doses in 24 hours count as 1 rescue			
14	Alcoholism, drug abuse or drug addiction within the past 2 years	Medical history, interview	Y / N	Y / N
15	Current felbamate with less than 18 months of continuous exposure	Medication history with start date	Y / N	Y / N
16	Current use, within 1 month of Visit 1, of diazepam, phenytoin, phenobarbital or metabolites of these drugs	Medication list. Ask the monitor how prodrugs such as primidone and fosphenytoin are treated	Y / N	Y / N
17	Vigabatrin now or within the past year. Any earlier vigabatrin use needs documentation of no clinically significant vigabatrin-associated abnormality on visual perimetry	Medication history, perimetry report	Y / N	Y / N
18	Status epilepticus within 3 months of Visit 1	Medical history, emergency and hospital records	Y / N	Y / N
19	History of 1 serious drug-induced hypersensitivity reaction, including Stevens-Johnson	Allergy list, medical history	Y / N	Y / N

#	Criterion	Verify against	Visit 1	Visit 3
	syndrome, toxic epidermal necrolysis or DRESS, or any drug-related rash requiring hospitalization			
20	AED-associated rash that involved conjunctiva or mucosae, or more than one maculopapular rash that required discontinuation	Allergy list, AED history	Y / N	Y / N
21	Creatinine clearance under 50 mL/min by the Cockcroft-Gault equation	Calculation on the lab worksheet from Visit 1 creatinine, age, weight and sex	Y / N	Y / N
22	Absolute neutrophil count under 1500/ μ L	Central lab report	Y / N	Y / N
23	Clinical or ECG evidence of serious cardiac disease, including ischemic heart disease, uncontrolled heart failure and major arrhythmias, or relevant replicated QT changes: QTcF under 340 msec, over 450 msec in males or over 470 msec in females	Triplicate ECG at Visit 1 and ECG at Visit 3, read and signed by the investigator. Cardiac history	Y / N	Y / N
24	Platelet count under 80,000/ μ L	Central lab report, medication list	Y / N / NA	Y / N / NA

#	Criterion	Verify against	Visit 1	Visit 3
	in a subject treated with valproate			
25	C-SSRS Baseline/Screening version: Yes to ideation question 1 or 2 in the past 6 months, or Yes to any suicidal behavior question in the past 2 years	Completed C-SSRS	Y / N	Y / N
26	More than 1 lifetime suicide attempt	C-SSRS, medical history	Y / N	Y / N
27	Participation in another trial of an investigational product or device within 30 days of screening, or longer if local rules require	Interview, medical record	Y / N	Y / N
28	Current use, within 1 month of Visit 1, of clopidogrel, fluvoxamine, amitriptyline, clomipramine, bupropion, methadone, ifosfamide, cyclophosphamide, efavirenz or natural progesterone	Medication list reviewed drug by drug	Y / N	Y / N
29	History of a positive antibody or antigen test for hepatitis B, hepatitis C or HIV	Medical record, interview. The protocol does not require new testing	Y / N	Y / N

#	Criterion	Verify against	Visit 1	Visit 3
30	Congenital short QT syndrome	Medical history, ECG	Y / N	Y / N
31	Any previous exposure to YKP3089	Interview, prior trial records	Y / N	Y / N

Baseline seizure count, Visit 3

This table is the source for inclusion criterion 6. Count only simple partial seizures with a motor component, complex partial seizures and secondarily generalized seizures; nonmotor simple partial seizures do not count, and neither do seizures on Day 1.

Measure	Requirement	Result
Baseline start date (Visit 1) and end date (day before Visit 3)	8 weeks	
Qualifying seizures, weeks 1 to 4	At least 3	
Qualifying seizures, weeks 5 to 8	At least 3	
Total qualifying seizures	At least 8	
Longest seizure-free interval, in days	25 or fewer	
Diary days with missing entries	Record the number and reason	
Rescue benzodiazepine uses during baseline	No more than 1	
Concomitant AED doses unchanged since Visit 1	Yes	
Diagnostic Review Form approved and signed by the independent reviewers, date	Before randomization	
Counted by, and checked by a second person	Two sets of initials and dates	

Have a second person recount the diary. A miscounted baseline is an easy way to randomize an ineligible subject.

Extension entry, Visit 9

Re-answer every inclusion and exclusion row except inclusion 6, using Visit 9 findings. Add the result here.

Check	Requirement	Result
Completed the 12-week double-blind maintenance phase	Yes	
All inclusion criteria met, seizure frequency aside	Yes	
No exclusion criterion met	Yes	
Subject chooses to enter the extension, documented	Yes	

Sign-off

The investigator's Visit 3 signature time must come before the IWRS randomization time. Auditors compare the two.

Who	Statement	Printed name	Signature	Date and time
Coordinator	I verified each row against the source listed			
Investigator, Visit 1	The subject may enter the baseline period			
Investigator, Visit 3	All inclusion criteria are met and no exclusion criterion is met. The subject may be randomized			
Investigator, Visit 9	The subject still meets all criteria except seizure frequency and may enter the extension			